

CITATION: *Herrington v. Brewer et al*, 2022 ONSC 2852
COURT FILE NO.: 3076/17
DATE: 20220516

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:

PAUL A. HERRINGTON

Plaintiff

)
)
) Michael Ellis and Robert Seredynski,
) Counsel for the Plaintiff
)
)

– and –

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)
) WALTER W. BREWER, ONTARIO
) CORPORATION NUMBER 1623687
) operating as NEW HORIZONS CAR AND
) TRUCK RENTALS LTD., JENNIFER M.
) STEWART and ONTARIO
) CORPORATION NUMBER 624252
) operation as WESTERN ASSURANCE
) COMPANY
)
)

Defendants

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) David Visschedyk, Counsel for the
) Defendants
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) **HEARD:** November 15 – 18, 22, 23, 25, 26
and December 2, 2021, by ZOOM

C.M. SMITH J.

REASONS FOR JUDGMENT

OVERVIEW

[1] This is a claim for damages arising from a motor vehicle accident which occurred on December 27, 2015.

[2] The defendants have accepted liability for the collision. What remains to be determined is the issue of causation, the quantification of damages, and whether or not the injuries meet the threshold set out in s. 267.1 of the *Insurance Act*, RSO 1990, c.I.8.

[3] The plaintiff is a 60-year-old gentleman who has spent his life working as an auto mechanic. He is the owner/operator of an auto repair shop.

[4] On December 27, 2015, the plaintiff was returning home from a road trip in his automobile. He was first in a line of vehicles stopped at a red light. A van operated by one of the defendants rear-ended the vehicle immediately behind the plaintiff which in turn was propelled into the rear of the plaintiff's vehicle. As a result of the impact the plaintiff's vehicle was pushed forward into the intersection.

[5] The plaintiff reported neck and back pain immediately after the collision. He elected to return home rather than attend at the hospital.

[6] After three days he attended at his family physician's office complaining of pain in his left elbow.

[7] The pain in his left elbow has interfered substantially with the plaintiff's ability to do the work of an auto mechanic.

[8] The plaintiff therefore seeks general damages for pain and suffering, pecuniary damages for loss of earnings, loss of competitive advantage/loss of earning capacity, as well as present and future costs of care including housekeeping and handyman services. In total the plaintiff seeks approximately \$1,250,000 in damages.

[9] The defendant takes the position that the injury in question was not caused by the accident but rather, was the result of repetitive strain caused by a lifetime of lifting heavy objects in the course of working as an auto mechanic. The defendant submits that if the injuries are found to have been caused by the accident, then general damages should be no more than \$20-\$30,000.

[10] The defendant also submits that the plaintiff has successfully mitigated his financial losses and is now making as much, or more, annual income as he was at the time of the accident. The defendant further submits that any cost of future care should be limited to the cost of the treatment suggested by the defendant's medical expert.

ISSUES

1. Were the plaintiff's injuries caused by the MVA?
2. If so, what are the appropriate types and *quantum* of damages?
3. Should an adverse inference be drawn from the fact the plaintiff did not call some of his treating health professionals to give evidence in the trial?

4. Has the plaintiff established on a balance of probabilities that he meets the exceptions set out in section 267.1 (3) and (5) of the *Insurance Act*, aka, “the threshold”?

ISSUE #1: Were the plaintiff's injuries caused by the MVA?

The evidence on the issue

1. The plaintiff

[11] The plaintiff testified that at the moment of impact he was holding onto the steering wheel with his left arm and had his right hand resting on the shifter. He described a sensation of a pulling movement in his left arm. Immediately after the collision he felt some discomfort in his neck and back which subsequently cleared up. He chose not to seek medical attention, however, over the course of the next three days he began to experience significant pain and discomfort in his left elbow, so he went to see his family doctor.

[12] The plaintiff had no issues with his left arm prior to the collision. He also had no prior difficulty performing the daily work of an automobile mechanic or engaging in his numerous leisure activities. Post collision he has been unable to do any of those activities notwithstanding his best efforts.

[13] The plaintiff continues to experience pain some six years after the accident.

[14] The plaintiff struck me as a straightforward, rather shy, plain-spoken individual. It is clear from his evidence, and the evidence of his employees, customers, and family members, that he has always been a hard worker who enjoyed working in his chosen trade and using his handyman skills. There is nothing in the evidence before me to suggest the plaintiff is malingering, dissembling, or exaggerating the nature, extent, and impact of his injury. I accept his evidence.

2. The course of treatment

[15] The Joint Document Brief filed by the parties contains the clinical notes and records of the plaintiff's family physician, his physiotherapist, as well as his psychologist, physiatrist, and chiropractor. It also contains a treatment chronology setting out all of the plaintiff's attendances for treatment by health professionals between November 7, 2013, and January 28, 2021. During that period of time the plaintiff attended more than 160 appointments with his health professionals only 8 of which predated the collision. He attended over 100 chiropractic and massage therapy sessions alone.

The medical experts

1. Dr. Su

[16] Dr. Su was the plaintiff's treating physician for this injury. He was qualified on consent as a participant expert in the field of sports medicine.

[17] Dr. Su testified that the plaintiff's injury consisted of a flexor tendon tear which resulted in ongoing pain and disability. He testified that the injury was caused by the collision. He also testified that in his opinion the condition will probably be permanent.

[18] Dr. Su described the plaintiff as a stoic individual who, in his view, is likely not exaggerating the nature and extent of his discomfort and disability. In Dr. Su's words the plaintiff "can barely pick up a coffee" using his left arm.

[19] In Dr. Su's opinion the plaintiff has medial epicondylitis with atrophy present. The atrophy indicates an injury. An injury stops an individual from using the limb in question which in turn causes the atrophy.

[20] Dr. Su opined that the tear would explain the atrophy. He describes the size of the tear as "moderate".

[21] Doctor Su used steroid injections to treat the arm. He did not use collagen due to the expense.

[22] Dr. Su noted that the plaintiff's right elbow also has chronic but moderate tendinosis. He did note though, that there were different results in the medial flexor in the right arm as opposed to in the left arm. Dr. Su testified that age, trauma, and overuse are the usual causes of this kind of condition but went on to say that something has to happen to make it actually symptomatic. He suggested that a sudden strain such as being pushed forward or rearward, or as he described it, "awkward forward or rearward traction", could be such a mechanism. Dr. Su suggested that the forces in the collision could cause such traction.

[23] Dr. Su also agreed in cross examination that the condition can be caused by repetitive bending and straightening however, he queried why the plaintiff would have such problems manifesting themselves first in his left arm when he is right hand dominant.

2. Dr. Axelrod

[24] Dr. Axelrod is an orthopedic surgeon who was retained by the plaintiff to review his case and to give expert opinion evidence in this trial about his injury and the prognosis for the future.

[25] Doctor Axelrod was qualified on consent as a expert in orthopedic surgery with a specialty in upper limb reconstruction.

[26] Doctor Axelrod has spent his professional life dealing with upper extremity injuries as a staff surgeon with the Sunnybrook Hospital in Toronto. In addition to being responsible for an active surgical practice, Dr Axelrod is also a full professor at the University of Toronto medical school. He has published extensively in the area of orthopedics. He also does medical legal work however, he emphasized that this is but a small part of his practice. Perhaps more importantly, he ensures that he does an equal amount of medical-legal assessment work for both plaintiffs and defendants.

[27] Dr. Axelrod has dealt with many cases involving the same injury suffered by the plaintiff.

[28] Dr. Axelrod is of the view that the plaintiff's injuries were caused by the collision. He points to the fact that the plaintiff had no pre-existing elbow pain prior to the collision and suggested that "the temporal relationship is pretty hard to dispute".

[29] Dr. Axelrod also testified that due to the nature of the plaintiff's work the area of his medial epicondyle is more susceptible to an injury of this nature.

[30] Doctor Axelrod reviewed the report of the defendant's expert, Dr. Boynton, prior to trial. He took issue with Dr. Boynton's suggestion that there was a lack of symptoms after the accident. Doctor Axelrod points to the fact that the plaintiff complained of pain in his elbow shortly after the accident which is what caused him to consult with his family physician three days later.

[31] With the respect to the long-term prognosis for the plaintiff, Dr Axelrod notes that most patients with this type of condition improve over a period of 1 to 2 years. He goes on to say the fact the plaintiff continues to have significant symptoms in his elbow leads him to conclude that the plaintiff's condition will not improve much further.

[32] Doctor Axelrod does suggest that further treatment involving the use of an elbow strap, continued physiotherapy, stretching exercises and perhaps use of adjuvant injections, such as platelet rich plasma, may be of assistance to the plaintiff. He does not believe that surgical treatment is likely to be successful.

[33] Dr Axelrod is of the view that the plaintiffs elbow injury is a permanent condition and a permanent and serious impairment of an important physical function.

3. Dr. Boynton

[34] Doctor Boynton was retained by the defence to review the plaintiff's case and to give her expert opinion as to the nature and cause of the plaintiff's injuries. She was qualified on consent to give expert opinion evidence in the field of orthopedic medicine. Over the plaintiff's objection I allowed the further qualification of a specialty in upper extremities based on Dr. Boynton's training in shoulder surgery and sports medicine as a fellow at UCLA, and her subsequent work and experience working with professional athletes.

[35] In the course of the qualification questions Dr. Boynton agreed that other than mandatory annual continuing medical education she has taken no professional training since 1993, has held no hospital appointments in Canada for 15 years, and last performed surgery in 2007 or 2008. She also agreed that she has published no peer reviewed articles for over 20 years and that the focus of her practice is now on rehabilitation rather than surgery.

[36] Doctor Boynton also testified that she does approximately 100 medical legal assessments per year, 90% of which are done on behalf of defendant insurance companies. She said that plaintiffs do not tend to call her.

[37] Doctor Boynton testified that the objective medical evidence she reviewed in this case did not suggest the traumatic tear was caused by the collision. She outlined the specific medical symptoms she would expect to see from such a traumatic tear which included immediate pain, swelling, and sometimes bruising, and loss of mobility with associated weakness. She noted that none of those symptoms were found in this case. She was specifically struck by the fact that the plaintiff was still able to use his arm after the accident.

[38] Dr. Boynton opined that the most likely explanation for the injury is a pre-existing minor tear of the common flexor tendon that was asymptomatic as the plaintiff denied having any symptoms prior to the collision.

[39] In cross examination Dr. Boynton reiterated her position that the tear of the tendon was a pre-existing condition. However, she also agreed that it was rendered symptomatic by the collision.

[40] Doctor Boynton was also of the view that the greatest issue facing the plaintiff is the atrophy in his left forearm muscle. Dr. Boynton suggested this can best be dealt with by way of strengthening exercises involving progressive, eccentric, loading of the muscle which will allow it to “heal, hypertrophy and build tolerance to load” so that the plaintiff can return to his usual activities.

[41] In her written report, filed in the Joint Book of Authorities, Dr. Boynton suggests she would not limit the plaintiff in his ability to perform his regular daily activities, including participation in recreational activities, in fact she would encourage him to do so. She would also not restrict the plaintiff in his ability to perform housekeeping and home maintenance activities. Lastly, she opined there is no role for surgery, or treatment by injections, as the main issue here is related to disuse and improper posture.

Biomechanists

1. Dr. Kumar

[42] Dr. Kumar is a world-renowned bio-mechanist who was retained by the plaintiff to review his case and provide expert opinion on the cause of his injury. He has published over 30 papers on the effects of rear end collisions on body tissues. He has taught at the university level in the fields of engineering and medicine. He has received the Order of Canada for his contributions to his field

of study. He is professor emeritus from the University of Alberta as well as a professor and ex director of physical medicine at the University of North Texas Health Science Centre.

[43] Dr. Kumar was qualified without objection as an expert in biomechanics, being the study of the effects of force on the body.

[44] Dr. Kumar testified that the plaintiff's grip on the steering wheel at the time of the collision would have caused "a sudden and forceful wrist flexion and a rapid forceful extension of the elbow." That in turn would have caused "the elbow flexor muscles to contract vigorously" placing an "instantaneous load on the flexor tendon and medial collateral ligament complex". That force would have "exceeded the viscoelastic tolerance limit of the flexor tendon in the medial collateral ligament" which, in his opinion, is what caused the plaintiff's flexor tendon to tear.

[45] Dr. Kumar specifically took issue with Dr. Boynton's theory of that the tear was caused by degeneration. Dr. Kumar suggests that degeneration alone does not cause disruptions of any tissue, but rather lowers the injury threshold such that when the tissue is subjected to stress beyond its reduced tolerance limit a rupture can result.

[46] Dr. Kumar was also critical of the human volunteer studies involving low velocity impacts cited by the defendant's expert, Mr. Kodsí, in his engineering bio-mechanics report. Dr. Kumar suggests that his own experience conducting such similar tests, which have led him to publish some 35 research papers in the area in peer reviewed international journals, lead him to conclude that the methodology is flawed due to the fact that investigators, often used as subjects, know how to position themselves so as to mitigate the risk of injury. More importantly, the subjects are typically young, strong males who are intimately aware of the procedure of the experiments and know what to expect. He suggested the fact that these studies have not led to any elbow injury in the subjects is because none of the studies cited in the report used a body configuration similar to that of the plaintiff or used subjects of more advanced age who have undergone some degree of degeneration.

[47] Dr. Kumar also noted the plaintiff is an older person with a lower injury threshold due to degenerative changes in his tissues. He noted that at the moment of impact the plaintiff was seated in a relaxed manner with his left hand between the spokes of the steering wheel while he waited for the traffic light to change. In Dr. Kumar's opinion that is not the typical driver posture used in the tests which is both hands holding the steering wheel. In Dr. Kumar's opinion the totality of these circumstances made the plaintiff more vulnerable to injury due to sudden impact.

2. Ms Avery Chakravarty

[48] Ms. Chakravarty gave evidence for the defence. She was qualified on consent as an expert in the field of accident reconstruction with a specialty in biomechanics.

[49] Ms Chakravarty has a Bachelor of Engineering degree and a Masters of Applied Science which focused on mechanical engineering. She took courses on the biomechanics of injury as well as in current topics involving orthopedic biomechanics. She is not affiliated with any university in

a teaching capacity, nor has she published any papers on the effects of low-speed collisions on body tissues.

[50] In the course of her testimony Ms Chakravarty testified that there have been hundreds of reconstruction studies of low-speed accidents done, many of which used live subjects. She said that she was unaware of any instance where a subject involved in any of those tests suffered injuries of the type described by the plaintiff.

[51] Ms Chakravarty agreed with the suggestion that the live crash test studies involve individuals who know that an impact is about to occur, which is knowledge which could affect the results of the study.

[52] In cross examination Ms Chakravarty did agree with counsel's suggestion that if the plaintiff had maintained his grip on the steering wheel, it would mean more tensile force in his arm. Further, if the plaintiff's body was propelled backward, it would mean further increased force on his arm and body which could result in extension of the elbow and wrist flexion. She did agree that flexor muscles would contract in such a scenario, which in turn would increase the likelihood of damage.

[53] Ms Chakravarty has done no specific research on elbow injuries. She did agree with counsel's suggestion that the plaintiff's work history might make him more vulnerable to such an injury.

Positions of the parties

The plaintiff

[54] The plaintiff submits the preponderance of evidence in this matter establishes that the injury in question was caused by the impact. He points to the MRI scan, and the radiological report that flowed from it, which confirmed that there was a tear in his left medial epicondyle.

[55] The plaintiff also points to the evidence of Doctor Su, his treating physician, and the evidence of Dr. Axelrod, the plaintiff's expert medical witness, both of whom are of the opinion that the collision caused the injury to the left elbow and that the condition will be permanent.

[56] The plaintiff also contends that the biomechanical evidence presented during the course of the trial established that the plaintiff's injury was caused by the accident.

[57] Lastly, while not agreeing with Dr. Boynton, the plaintiff nevertheless submits that her evidence to the effect that the tear was a pre-existing condition made symptomatic by the collision makes this a classic thin skull case.

The defendant

[58] The defendant submits that the plaintiff did not tell anyone about gripping the steering wheel until he testified in this matter. The defendant therefore submits that the opinions of the plaintiff's experts, Doctors Su, and Axelrod, as well as the bio-mechanist, Dr. Kumar, are based on assumptions they made which flow largely from the temporal relationship between the complaint and the accident.

[59] The defence is not taking the position that the plaintiff is lying or is otherwise unworthy of belief. Rather, the defence simply suggests that the evidence does not support the position that the plaintiff asserts and that he may be exaggerating his complaints and limitations, consciously or unconsciously, to support his position that he is gravely injured.

[60] The defendant also submits that Dr. Boynton, the defence orthopedic expert, testified that 95% of individuals who suffer this type of injury recover fully provided they use the appropriate strengthening exercises. She said this was a tiny tear that is very common and very treatable. Dr. Boynton focuses her medical practice on providing recovery-based rehabilitation. The defence submits that as the plaintiff did no such strengthening exercises, he has therefore failed to properly mitigate his damages.

[61] Lastly, with respect to the plaintiff's thin skull argument, it is the defendant's position that this is in fact a crumbling skull case where the plaintiff would likely have had the same issues without the collision.

Relevant legal principles

[62] The test to be applied in the determination of causation continues to be the traditional "but for" test. The plaintiff has the onus of proving, on a balance of probabilities, that "but for" the negligence of the defendant the injury would not have occurred: see *Resurface Corp. v. Hanke*, 2007 1 SCR 333.

[63] In the case of *Clements v. Clements*, [2012] S.C.J. No. 32, the Supreme Court of Canada reaffirmed that notion and found further that the test must be applied in "a robust, common-sense fashion."

[64] The SCC has also addressed the so called "crumbling skull" notion. It held that before a damage award can be reduced because of the crumbling skull issue there must be evidence from physicians about the likelihood that a similar injury would have been suffered by the plaintiff had tortious conduct not occurred: see *Athey v. Leonati* 1996 1 SCR 333.

Application of the relevant legal principles regarding causation to the facts of this case

[65] I have no reason to believe, or even suspect, that the plaintiff is malingering or dissembling when he describes the ongoing pain and discomfort he has been suffering since the date of the accident.

[66] Dr. Su was very clear in his evidence that in the course of treating the plaintiff for this injury he came to the conclusion that the injury could only have been caused by the collision and that it was permanent in nature.

[67] To be sure, Dr. Su gave his evidence with a rather disdainful air and was openly scornful of some of his professional colleagues. However, through the course of his testimony his expertise became readily apparent. He is clearly a very accomplished physician, as evidenced by his curriculum vitae. He was also the plaintiff's treating physician for this injury which makes him a participant expert. As such, his familiarity with the plaintiff's condition significantly exceeds that of the other health professionals who testified in this matter. In my view, that makes Dr. Su a very compelling witness.

[68] Dr. Axelrod was also a very compelling witness. His CV is replete with experience, publications, accomplishments, and honours. He gave his evidence and opinion in a very straightforward, plainspoken, down to earth manner, a manner which in my view always enhances the credibility of a witness. I was also struck by the fact that Dr. Axelrod endeavors to share his experience and expertise equally with both plaintiffs and defendants who seek his assistance with assessments. That suggests to me that Dr. Axelrod is fair minded.

[69] Dr. Axelrod, like Dr. Su, believes the plaintiff's injury was caused by the collision and that the effects of the injury are likely permanent in nature.

[70] Dr. Boynton took a slightly different approach to this matter. She does not believe that the tear was caused by the collision. Instead, she is of the view that the tear was a pre-existing, asymptomatic injury rendered symptomatic by the collision. She also does not believe that the consequences of that tear are permanent but rather, that the consequences could be eased, or possibly eliminated altogether, by a course of stretching and strengthening exercises.

[71] There are a number of reasons why I choose not to accept Dr. Boynton's assessment regarding the expected duration of the plaintiff's pain, discomfort, and disability, which are as follows:

- Between the date of the accident and May 2016 the plaintiff attended over 40 physiotherapy sessions without any appreciable long-term results.
- Between October 22, 2018, and July 8, 2020, the plaintiff attended over 60 chiropractic/massage therapy sessions, again without any appreciable long-term results.
- Notwithstanding Dr. Boynton's assertion that "there are many ways that he would be capable of modifying how he does his activities as a mechanic to allow him to continue to work" the evidence of the plaintiff, and his employees, family, friends, and customers, is that the plaintiff is simply no longer able to do the work of a mechanic, nor has he been able to since shortly after the collision.

- Dr. Su was the plaintiff's treating physician throughout his "recovery" from this injury. In my view that gives Dr. Su a deeper, more nuanced, understanding of the plaintiff's condition than is the case with the other medical witnesses.
- Dr. Su asserts that the plaintiff can barely pick up a cup of coffee with his left arm.
- Both Dr. Su and Dr. Axelrod are of the opinion that the plaintiff's condition is permanent.
- Dr. Boynton spends a considerable amount of her time doing medical legal assessments, approximately two or three per week. That work generates a substantial portion of her annual income. She does not attempt to balance that work as between plaintiffs and defendants. According to Dr. Boynton approximately 90% of those assessments are done at the request of defendant insurance companies. That gives rise to a reasonable perception of bias.

[72] I am also unable to point to any evidence from the physicians in this matter that satisfies me that this injury, if it was pre-existing, would likely have developed notwithstanding the collision. I am therefore not prepared to find that this is a crumbling skull case.

Conclusion on Issue #1

Causation

[73] I am satisfied that the plaintiff has established, on the balance of probabilities, that the plaintiff's current state of disability and pain was caused by the collision.

Are the injuries permanent?

[74] Based on the evidence that I prefer, being the evidence of Dr. Axelrod and Dr. Sue, I am also satisfied that the plaintiff's condition is permanent in the sense that it is going to last indefinitely, rather than having a predictable, defined end.

ISSUE #2: What are the appropriate types and *quantums* of damages?

[75] The plaintiff seeks damages under a number of heads including general damages for pain and suffering and special damages for pecuniary losses. I will consider each in kind.

[76] The onus is on the plaintiff to establish past losses on the balance of probabilities. With respect to future losses, such as those sought by the plaintiff for future income losses, the burden is lessened such that the plaintiff need only establish that there is a real possibility, as distinct from a probability, that such losses will occur.

General Damages for Pain and Suffering

Positions of the parties

1. The plaintiff

[77] The plaintiff submits that the appropriate range for an award of general damages for pain and suffering in this case is in the range of \$80,000 to \$120,000. He specifically suggests an award in the middle of that range in the amount of \$100,000.

[78] In support of that position the plaintiff points to the fact that he continues to experience pain everyday. While the degree of pain varies with his particular daily activities the fact remains that it never goes away.

[79] In particular, whenever the plaintiff tries to use his left arm, for example to lift something, push against something, or pull against something, he will have pain in the nature of an electrical shock. Once triggered that pain can continue for days.

[80] As a result of the pain, the plaintiff finds himself having to make a choice each day between dealing with the pain or foregoing the daily activities in which he has engaged every day of his working life. Those activities include both work and leisure activities.

[81] The plaintiff now must ask for help from others in order to complete tasks that he used to undertake himself. For example, his neighbor now does his snow removal, his coworkers now act as his hands in his workplace, and the plaintiff now stays indoors at his weekend home rather than engaging with and helping his neighbors.

[82] He has had to forego fishing, golfing, and boating which were all activities that he pursued avidly without difficulty prior to his injury.

[83] The plaintiff submits that whereas he was once a master mechanic, a master handyman and completely independent, he is now reliant on others to do the things he used to do without a second thought. In the plaintiff's submission this a loss of independence, which has been devastating for him.

[84] In the plaintiff's submission when considering a general damages award it is important to look beyond the injury itself and instead focus on its impact on the individual. He posits the case of a fractured pinky finger. For the average individual that would be a relatively insignificant and passing injury. However, as the plaintiff submits, for a concert pianist such an injury would be devastating.

[85] Counsel for the plaintiff suggests comparing the impact of the plaintiff's injury to that of a concert pianist is entirely appropriate given that this relatively minor injury, a small tear to the muscles in the left elbow, has had such a profound impact on the plaintiff's daily life and his ability to engage in the activities that he formerly enjoyed.

[86] The plaintiff also submits he will have to continue to seek treatment for that pain indefinitely, given the finding of his doctors that it is unlikely he will ever improve in any significant way.

2. The defence

[87] The defendant suggests an award of general damages in the range of \$20,000 to \$30,000.

[88] Counsel for the defendant acknowledges the plaintiff experienced pain as a result of this injury. However, he submits the continuing pain is not necessarily accident related. Rather, the defendant submits the ongoing pain is a result of the plaintiff's decision not to engage in the course of treatment suggested by Dr. Boynton, being stretching and strengthening exercises.

[89] As I understand the defendant's argument, it is acknowledged that the plaintiff experienced a degree of pain and discomfort from the date of the injury. However, the award of damages for that period of pain should be curtailed to a substantial degree such that it is limited to the pain and discomfort that would have been experienced between the date of the injury for a reasonable period of time to allow for the plaintiff to engage in, and benefit from, the course of treatment suggested by Dr. Boynton.

The relevant legal principles

[90] The plaintiff has supplied me with a number of authorities regarding damage awards for cases involving similar injuries. In my view, the following three cases most closely mirror the circumstances of this case.

[91] In the case of *Tennant v. Fariba*, 2013 ONSC 1676, a 47-year-old man who was a self-employed flooring installer sustained injuries to his arm, shoulder, lower back, and knee in a motor vehicle accident. At the time of trial, he continued to experience chronic pain disorder, stiffness and a lack of strength and sensation in his right arm. He testified that the pain was constant from his right hand all the way up to his right shoulder and that he struggled to bend or lift his arm. Upon his return to work he found he was obliged to utilize services of subcontractors as he was unable to perform the work himself. He developed depression which affected his social and recreational life. The trial judge in that case awarded him that the sum of \$65,000 in general damages which adjusted for inflation would be \$76,044.72.

[92] In the case of *Stroud v. Nicholson*, 2005 CarswellOnt 3997 (ONSC), a 50-year-old, female registered nurse sustained long-term injuries from an accident which included left arm pain, left shoulder pain, neck pain, reduced motion in her arm, tennis elbow and headaches. After a recovery period of about a year she returned to work only to find that the functional limitations imposed by her injuries prevented her from carrying out her employment duties. She was obliged to retire. The trial judge in that case found her injuries meant that she would continue to experience pain, fatigue, discomfort, limitation of movement, and work restrictions indefinitely, thereby substantially diminishing her enjoyment of life. In that case the damages award was \$125,000, which when adjusted for inflation equates to the sum of \$166,705.28.

[93] In the case of *Ashburn v. Storrey*, 2019 ONSC 6486, a 61-year-old retired female sustained injuries which led to chronic pain in her neck, shoulder, lower back, and hip. She was left with very limited use of her left arm as well as depression and insomnia related to chronic facial pain. She had been a homemaker and was no longer able to care for her husband and children. In that case the trial judge awarded her the sum of \$62,500 for general damages which, when adjusted for inflation, equates to \$65,840.

[94] From those cases I conclude that the appropriate range for an award of general damages in this case, which involves certain similar consequences, is somewhere between \$60,000 and \$166,000. The suggested range of the plaintiff appears to be in line with those authorities.

Application of the relevant legal principles to the facts of this case

[95] The plaintiff has experienced a substantial amount of pain and suffering as a result of this accident. Most of that has been enumerated elsewhere in these reasons. However, I do see the following as being the key points:

- As a result of the accident the plaintiff suffered a partial tear of the common flexor tendon insertion which led to a diagnosis of medial epicondylitis.
- His doctors have concluded that his condition is permanent and will likely not improve.
- He has developed atrophy in the muscles of his left arm.
- His course of treatment has involved over 160 medical appointments which included physiotherapy, psychotherapy, chiropractic/massage therapy, and steroid injections.
- Notwithstanding all of that treatment the plaintiff is still left with substantial pain and significantly diminished use of his left arm.
- The plaintiff is no longer able to fully engage in the duties of his chosen trade of auto mechanic.
- The plaintiff is no longer able to engage in his preferred leisure time activities which involved handyman work, golf, boating, fishing, and socializing with his family, friends, and neighbors.
- He has sold his boat and weekend property.
- He has developed an adjustment disorder with anxiety.
- He can no longer perform routine housekeeping and outdoor maintenance tasks.
- He has had to hire staff to assist him at his place of business.

- His ability to engage in volunteer activities has been curtailed.
- The plaintiff is a man who described his pre-accident life as being akin to that of the “Energizer Bunny.” His family members now describe him as depressed, sad, having gained weight, and spending his time sitting on his lazy boy and watching TV.

[96] In light of all of those factors I find the plaintiff’s suggested range of general damages to be quite reasonable in the overall circumstances of this case.

[97] I am also satisfied that the plaintiff has met his onus and has proven, on the balance of probabilities, that the injuries he suffered in the collision have resulted in the pain and suffering enumerated above.

[98] While the effects of the plaintiff’s injury are on a par with the effects of the injuries in the authorities cited, the injuries suffered by the plaintiff are not as extensive as those suffered by the other plaintiffs. Those cases each involve plaintiffs who suffered multiple injuries. That is a significant difference which suggests an appropriate award would be at the lower end of the range.

[99] I find the facts of the *Tennant v. Fariba* case most comparable to the plaintiff’s situation. Both cases involve loss of strength and sensation in an arm which included constant pain and difficulties lifting. The impact of the injury was also similar as both plaintiffs were obliged to change their work habits and learn to rely on the assistance of others to perform the work of their respective trades.

[100] The plaintiff in the *Tennant* case was awarded general damages of \$66,000. In my view that is an appropriate amount to award the plaintiff in this case, after adjusting for inflation. I therefore award the plaintiff general damages in the amount of \$75,000.

Special Damages for Pecuniary Losses

(a) Loss of Earning Capacity/Competitive Advantage

[101] Future income awards are based on the notion that an injured party should, to the extent possible, be placed back in the financial position they would have been in but for the injury. There are 2 distinct but nevertheless related concepts involved in such awards. First the notion of loss of future income as it relates to a probable loss of future earnings. That is a relatively straightforward calculation based on previous years income compared and contrasted with post accident income.

[102] The second related concept is the loss of competitive advantage caused by the injury. Where an injured person is no longer capable of performing the essential functions of their craft, trade, or profession, then they have suffered the loss of an asset. The question becomes the value of that asset. Once that is determined it is possible to assess the financial impact of that loss on the injured party by considering both current circumstances and future contingencies.

[103] In other words, such an award is an acknowledgement that because of the injuries suffered by the injured party, that person will no longer be able to compete as effectively in the marketplace as they were able to do prior to the injury. They are therefore said to have suffered a loss of competitive advantage.

Relevant legal principles

[104] A loss of competitive advantage is one component of future income loss. It is an award based on the unavoidable fact that because of the injuries suffered by the plaintiff his own capacity to earn money in his chosen trade has been taken away. He will no longer be able to compete in the marketplace with other able-bodied individuals who have comparable qualifications and experience: see *Andrews v. Grand & Toy Alberta Ltd.*, [1976] 2 SCR 229.

[105] Loss of earning capacity is an award based on the recognition that an injured party's capacity to earn money was an asset which has been taken away. The calculation is based on the level of earning which the injured party would likely have achieved during the period in which that income would have been sustained, subject to deduction for contingencies of life: see *D'Amato v. Badger*, [1996] 2 SCR 1071 at paras 53-59.

[106] The *D'Amato* case involved a 51-year-old male plaintiff who had a 50% interest in an auto body repair shop. As a result of shoulder injuries, the plaintiff could no longer perform the physical labour required for auto body repair but was able to continue in a managerial, supervisory role. Those new activities were described by the trial judge as minor even though post accident the plaintiff continued to receive the same salary from the business notwithstanding his changing responsibilities.

[107] The trial judge awarded the plaintiff \$290,000 for loss of competitive advantage based on the plaintiff's age, his literacy level, and the fact that he had spent his entire working life doing the kind of physical work in question. The BC Court of Appeal reduced that amount significantly. On appeal to the Supreme Court of Canada the decision of the trial judge was restored as the Supreme Court found there was no legal basis for interfering with the award. The Supreme Court also found that the Court of Appeal had ignored the possibility that business realities could compel the closure of the business in question which would mean the plaintiff would not be able to replace his income.

[108] Awards for loss of competitive advantage are often made without an accompanying award for loss of future earnings in circumstances where the injured party has returned to their pre accident level of income but nevertheless has a higher risk of being unemployed in the future due to factors caused by the accident. In such circumstances the difference between the present values of the pre accident probable income and the post accident probable income will be the amount typically awarded for loss of competitive advantage: see *Conforti (Re)*, 2012 ONSC 199 at para 45.

[109] Other courts have followed that approach most notably in the case of *Tennant v. Fariba*, 2013 ONSC 1676, and *Cerilli v. Ottawa (City)*, 2008 ONCA 2. In both cases the court made loss

of competitive advantage damage awards because it was found that each of the plaintiffs had suffered a loss of marketability due to their injuries. See also *Earl v. Lang*, 199 CanLII 18660 (ONCA), and *St-Prix v. Home Depot*, 2008 CanLII 115 (ONSC).

Application of the relevant legal principles to the facts of this case

[110] At the time of the accident in this case the plaintiff was 56 years of age. He had spent his life working as an auto mechanic. He owned his own auto mechanic shop where his duties involved working as a full-time mechanic. He employed an assistant who shared in that work. He had no plans to retire. His business enjoyed a steady pace.

[111] Following the accident, the plaintiff was unable to perform the work of a mechanic and is now relegated to administrative and supervisory work. He has been obliged to hire additional workers to take over his responsibilities and to allow the business to remain viable. Those new additional workers testified in this matter and confirmed that the plaintiff is incapable of doing the work of a mechanic.

[112] That fact was confirmed by Dr. Axelrod who opined that the plaintiff's injury interfered substantially with his ability to complete mechanics duties.

[113] The plaintiff continues to enjoy an income at or above the level he enjoyed at the time of the accident. However, his current employees appear to be willing to work as mechanics for an annual income between \$30,000 and \$40,000. That strikes me as being an inordinately low annual income, particularly in view of the evidence that I heard about there being a labour shortage involving skilled mechanics. Market forces will inevitably drive-up workers wage expectations which in turn will make it increasingly difficult for the plaintiff to find employees.

[114] The plaintiff's son-in-law and former employee, Mr. Jennings, opted to leave the plaintiff's employment for a position with a higher rate of pay at a car dealership. Such positions are apparently seen as much more desirable than equivalent positions in shops like the plaintiff's because of features such as higher pay, workplace benefits, better job security, more regular hours, and often, better working conditions.,

[115] Mr. Reid and Mr. Harvey also left the plaintiff's employ for similar reasons.

[116] Given the challenges imposed by his injury the plaintiff is at a distinct disadvantage in terms of competing in the mechanic's trade. Given the labour shortage there is a very real possibility that the plaintiff may one day find that he is unable to find skilled employees who he can rely upon to generate the income he currently enjoys. That in turn would bring the viability of his business into question which would have a negative impact on its value as the plaintiff's main asset. That would mean the plaintiff would be doubly disadvantaged as the value of his business, and the level of income it generates, would both be negatively affected.

[117] The plaintiff testified that he feels vulnerable because he cannot find new employees. He said “if one of my guys leaves I’m in trouble.” He acknowledged he would not hire himself. He said he fears for the future viability of his business.

[118] Based on the evidence of the plaintiff, his employees, and his doctor, I am satisfied on the balance of probabilities that the injuries suffered by the plaintiff have caused him to suffer a loss of competitive advantage.

What is the appropriate *quantum* of damages for the plaintiff’s loss of competitive advantage?

[119] The only evidence before me on this point comes in the form of a report prepared by Mr Ian Wollach who is a CPA and a partner at RSM Canada Consulting LLP. Mr. Wollach has extensive experience providing expert witness testimony in courts in various provinces and countries. He has provided expert witness testimony on over 190 occasions for plaintiffs, defendants, and insurers.

[120] Mr. Wollach was qualified on consent in this manner to give expert opinion evidence in forensic accounting with a specialty in quantification of damages and economic loss. The report he prepared at the behest of the plaintiff was made an exhibit in this manner and is found in the Joint Document Brief.

[121] In Mr. Wollach’s opinion, the fairest approach to this issue is to follow the methodology used by the Supreme Court of Canada in the *D’Amato* case and calculate the plaintiff’s losses on the following factors:

- the fair market value of his pre accident contribution to the business (ignoring distributions either motivated by tax planning or attributable to his investment in the business);
- the present value over the remainder of his projected pre accident working life: and
- reduced to reflect the fair market value of his post accident projected future contributions to the business.

[122] Mr. Wollach used that approach in calculating 3 separate scenarios regarding the plaintiff’s past and future income losses. The first two scenarios were based on the plaintiffs pre accident earning capacity, to normal retirement at age 68, based on his inflation adjusted average earnings from the business. Scenario one predicated that calculation on the plaintiff’s average earnings from the business in the years 2013 through 2015. Scenario two is based the calculation solely on the plaintiff’s earnings in 2015. Scenario three is based instead on inflation adjusted average income of full-time male auto mechanics per Statistics Canada, also through normal retirement age at 68.

[123] Mr. Wollach also assumed a 70% loss of current capacity in scenarios one and two because of the plaintiffs continued supervisory and administrative role in the business.

[124] I prefer scenario 1. It utilizes a broader range of the plaintiff's actual income as opposed to scenario 2 which is based on only one year's income or scenario 3 which focuses on the actual statistical earnings of the occupation of the plaintiff, as opposed to focusing on the actual situation of this particular plaintiff.

[125] In scenario 1 Mr. Wollach determined the plaintiff's past income loss through the start of the trial on November 15, 2021, to be \$231,477. He determined the plaintiff's present value of future income losses from November 16, 2021, to normal retirement age at age 68, as at November 15, 2021, to be \$340,118. The total of past and future income losses as of the start of trial in scenario 1 is therefore \$571,595.

[126] I award the plaintiff the sum of \$571,595 in damages for loss of earning capacity/loss of competitive advantage.

(b) Payments Made to Others

[127] At the time of the accident the plaintiff had one employee who was also an automobile mechanic. He and the plaintiff were able to handle all of the physical work in the shop as well as the administrative tasks. The evidence shows that the two were able to complete all such tasks in a timely manner and to keep up with the repair work.

[128] Following the accident, it quickly became clear to both the plaintiff and his employee that the plaintiff was no longer able to do the physical work required of automobile mechanic. Accordingly, a decision was made to hire another mechanic to do the work the plaintiff could no longer do himself.

[129] The plaintiff seeks an award of \$281,926 on account of the monies he has paid to this additional employee who he hired as a result of the accident. That sum consists of the total wages paid to that employee through the start of the trial, discounted to 70% per the *Insurance Act* and Mr. Wollach's evidence, together with the present day value of anticipated future wages through retirement at age 68.

[130] I decline to make that award.

[131] Like any injured party, the plaintiff had an obligation to mitigate his damages. He has done so and done it very well. Immediately after the accident he recognised the reality and impact of his injury whereupon he reorganised his business and hired an employee to do the work he could no longer do himself. He bills his customers for that employee's time at an hourly rate that covers that employee's wages and yields a profit for his business. That business of course belongs solely to the plaintiff and his wife.

[132] The plaintiff's business is now generating higher annual revenues than in the past which has led to a corresponding increase in the plaintiff's own annual remuneration such that he is now enjoying a higher standard of living. By taking on a supervisory role, and assuming responsibility

for the administrative tasks of the business, the plaintiff has freed his employees to spend their time doing the work that actually generates the revenues of the business.

[133] The monies the plaintiff pays his employees in wages each year is an investment he makes in order to generate his own income. The plaintiff has properly, and successfully, met his obligation to mitigate his damages. In such circumstances the award sought would be a windfall for the plaintiff.

(c) Loss of R.E.R. Revenues

[134] The plaintiff enjoyed a 20-year business relationship with Mr Ron Reinstra, the owner/operator of R.E.R. Automobiles, a used car dealership near the plaintiff's shop. Prior to the accident the plaintiff received an average of \$3000 to \$4000 per month in pure revenue from repair work done on Mr. Reinstra's automobiles.

[135] After the accident it very quickly became clear that plaintiff was not able to perform the work required by Mr. Reinstra in the same timely manner and to the same standard that came to be expected of him prior to the accident. As a result, Mr. Reinstra stopped sending work to the plaintiff resulting in a net loss to the plaintiff's business of \$42,000 per year for each of 2017, 2018 and 2019, for a total of \$126,000.

[136] In the course of his testimony Mr. Reinstra indicated that if the quality of the plaintiff's work had continued at the same level as before the accident, he would not have stopped sending him work. In that scenario he said he would be sending at least that much work to the plaintiff each year, if not more. He also advised that he now has so much of this sort of work to be done that he started sending the plaintiff work once again during 2020. That work generated revenues for the plaintiff of approximately \$18,000 in 2020 and \$16,500 in 2021 to the start of the trial, for a total of \$34,500.

[137] The plaintiff therefore contends that but for the accident he would have continued to enjoy those revenues.

[138] The plaintiff asserts that the total past loss of the R.E.R. revenues is \$160,500. The plaintiff submits that he is entitled to 70% of that amount, or some \$112,350, in accordance with the provisions of the *Insurance Act* and in accordance with the formula established by Mr. Wollach in his income loss report.

[139] The plaintiff also asserts that he is entitled to the present value of future losses of those amounts in the amount of \$18,000 per year through age 68 per Mr. Wollach's evidence. Using the present value discount factor supplied by Mr. Wollach of 5.59 that equates to a further sum of \$100,620 ($5.59 \times \$18,000 = \$100,620$).

[140] In total the plaintiff seeks \$212,970 ($\$112,350 + \$100,620 = \$212,970$) in damages as a result of those lost revenues.

[141] The defendant submits that while there was a minor revenue drop in 2017 after R.E.R. stopped sending work to the plaintiff the revenues that year were still higher than they were before the accident. The defendant suggests that means the plaintiff's company picked up more work from other sources. The defendant also submits that there is no evidence tending to suggest that the plaintiff's company could have handled the work that R.E.R. would have provided without an increase in expenses.

[142] The evidence before me shows that prior to the accident the plaintiff was handling the R.E.R. work in a timely, workmanlike manner such that Mr. Reinstra was happy to keep sending work, apparently on an ever-increasing basis.

[143] Whether or not the plaintiff could have continued to handle that work without hiring more staff is really a moot point. The fact is, Mr. Reinstra decided to take his work elsewhere following the plaintiff's injury. He has only recently started sending work the plaintiff's way because he now has more of such work than ever before. Mr. Reinstra was very clear about the fact that he would gladly have continued to send all of his automobile repair work to the Plaintiff.

[144] The evidence establishes quite clearly that the plaintiff enjoyed annual revenues from R.E.R. of approximately \$42,000 per year. As a result of his injuries the plaintiff was obliged to forgo that work entirely for three years from 2017 through 2019. The work resumed in a much-reduced capacity commencing in 2020. That is a significant loss by anyone's standards. That loss would not have occurred but for the injuries suffered by the plaintiff in the collision.

[145] I therefore find it appropriate to award the damages claimed in these circumstances, being the sum of \$212,970.

(d) Future Costs of Care

[146] Based primarily on the expert medical evidence before me I have concluded that the injury suffered by the plaintiff is likely permanent in nature. That being the case, the plaintiff will be obliged to attend for, and incur the expense of, various future treatments, likely for the rest of his life.

[147] The plaintiff is entitled to compensation for future potential costs provided there is a reasonable chance that those costs will need to be incurred. The plaintiff must show that there is a "real and substantial possibility" that such costs may be incurred: see *Ashley v. Leonati*, supra.

[148] The *quantum* of damages for the cost of future care is typically a matter of conjecture, prediction, and guesswork. Damages must be assessed at the time of trial so a court considering future damages must rely on whatever evidence is at hand regarding what the cost of such care is likely to be. There was some evidence before the court in this matter from the doctors regarding the likely cost of future treatments the plaintiff may have to obtain.

[149] The defendant pointed to the fact that the plaintiff acknowledged that he received money from his accident benefits carrier which he did not use to obtain the related treatment. That may well be, however, it has been held that plaintiffs are free to do with such sums of money as they like. It is not for this court to guess about what the plaintiff may or may not do with monies paid to him. The obligation of this court is simply to compensate the plaintiff for costs such as these: see *Andrews v. Grand & Toy Alberta Ltd.*, supra.

[150] Doctor Axelrod suggested that the plaintiff continue with physiotherapy treatments every other week for at least two years. The cost of that is estimated at \$75 a session which when multiplied by 52 (26 weeks x 2 years) results in a total expected outlay of \$3900.

[151] Dr. Su suggested synthetic collagen injections would likely be of value. Those injections are \$660 each for a further expense of \$1320.

[152] Both Dr. Su and Dr. Axelrod recommend 2 Platelet Rich Plasma injections. Those injections are \$1000 each for a total of \$2000.

[153] All recommended future care treatments total \$7220.

[154] In addition to the future care costs, I note that the plaintiff has to date spent some \$2683 for chiropractic treatments with Dr. Dorman. Consistent with the general approach to the question of accident benefit expenses the plaintiff should be compensated for that outlay.

[155] I therefore award the plaintiff a total of \$9903 ($\$7220 + \$2683 = \9903) for both care costs incurred to date as well as the estimated cost of future care treatments.

(e) Housekeeping and Handyman Duties

[156] A court is entitled to make an award for these types of losses where there is evidence that there is a real and substantial possibility that the plaintiff will be unable to perform all of his usual and necessary household work; see *Solanki v. Reilly*, 2021 ONSC 6694.

[157] The Ontario Court of Appeal has held that there are 3 scenarios in which the action, or perhaps more accurately the inaction, of a plaintiff constitutes evidence of a tangible loss of housekeeping capacity. They are as follows:

1. Where the plaintiff leaves all or part of the housework undone;
2. Where the plaintiff does some of the housework but does it with pain or difficulty and therefore inefficiently, and
3. Where the plaintiff's housework is actually done by third parties.

See: *McIntyre v. Docherty*, 2009 ONCA 448.

[158] In this case there is evidence that the plaintiff is unable to do snow removal and must rely on his neighbor for that service. There is also evidence that the plaintiff can no longer take the garbage out or do other such routine household chores.

[159] Perhaps more significantly is the evidence before this court of the plaintiff's inability to perform the significant handyman work that he has traditionally taken on, with some considerable proficiency. There is a significant amount of evidence before me of the plaintiff's accomplishments in that area prior to the collision. It is clear to me that he was always ready, willing, and able to take on any household repair or improvement task no matter how substantial or challenging.

[160] There is also evidence before me that the plaintiff was obliged to enlist the assistance of his next-door neighbors to repair his balcony railing, work that traditionally the plaintiff would have been only too happy to take on himself.

[161] There was also evidence from both the plaintiff and his wife that they have embarked on the project of re-doing their kitchen cabinetry. The cabinetry is estimated to cost approximately \$20,000. Labour to install the new cabinetry is estimated to cost a further \$20,000. That is labour that the plaintiff would traditionally have done himself. As a result of his injury he is therefore faced with the added expense in the amount of \$20,000 for labour for that job.

[162] This court dealt with a similar problem in the case of *Pelletier v. Ontario*, 2013 ONSC 6898. In that case the caregivers claim had not been formally articulated or advanced in the detailed manner that the court found was entirely fair to the defendants. However, the trial judge in that case was satisfied that the trial would remain procedurally fair even if that issue were to be addressed. The court explained that one of the principal functions of the court in ensuring substantive fairness to a defendant is to require that claims advanced are legitimate and reasonable. It found that the claim for attendant care well should not have taken the defendants by surprise and could have been anticipated.

[163] I am satisfied that there is a real and substantial possibility that the plaintiff will have to incur housekeeping and handyman costs, occasionally substantial in nature, in the future. In fairness to both parties I would award the plaintiff the sum of \$25,000 against those future costs, that sum being based on the estimate before me. The fact that I am unable to anticipate or perceive any other such costs means that the plaintiff will have to bear such costs should they arise, a fact which in my view balances the scales on this particular issue.

ISSUE #3: Should an adverse inference be drawn from the fact that the plaintiff did not call some of his treating health professionals to give evidence in the trial?

[164] In closing argument, the defendant pointed to the fact that the plaintiff did not call his family doctor, his psychologist, his chiropractor, or his physiotherapist to the stand to testify in this matter. The defendant submits that the plaintiff has a duty to prove their case but the only

treating doctor that they called was Dr. Su. The defendant urges me to draw an adverse inference against the plaintiff in that regard.

[165] The plaintiff points to the fact that he supplied the clinical notes and records of an assortment of his treating health professionals to the defendant well in advance of the trial. The following are the relevant clinical notes and records supplied to the defendant by the plaintiff, which are filed in the Joint Document Brief:

1. Clinical notes and records of the plaintiff's family physician, Dr. Gertler, from November 7, 2013, to April 13, 2018
2. Clinical notes and records of Dr. Su from December 27, 2015, to February 9, 2018
3. Disability certificate (OCF 3) from Grant Physiotherapy dated April 22, 2016
4. Clinical notes and records of Grant Physiotherapy from January 6, 2015, to May 25, 2016
5. Psychological report by Dr. Ana Bodner dated September 4, 2017
6. Clinical notes and records of Dr. Bodner & Associates January 17, 2018, to February 21, 2018
7. Physiatry Insurers Examination Report by Dr. Jogindra Singh dated July 11, 2016
8. Clinical notes and records of Dr. Lesley Corrin dated September 28, 2018
9. Decoded OHIP summary from December 27, 2012, to August 16, 2017
10. Psychological Insurer's Examination Report by Dr. Lamarche dated August 15, 2017
11. Clinical notes and records of Dormon Chiropractic Massage Therapy from October 22, 2018, to July 8, 2020
12. Updated clinical notes and records of Dormon Chiropractic Massage Therapy dated November 3, 2021
13. Chronology showing each and every attendance for medical treatment made by the plaintiff between November 7, 2013, and January 28, 2021

[166] The plaintiff submits that he did not call these witnesses as he was content to rely on their clinical notes and records as well as the *viva voce* testimony of Dr. Su. The plaintiff submits that to do otherwise would have unnecessarily lengthened the trial and would only have provided repetitive evidence to the court.

[167] The plaintiff further submits that the defendant was fully apprised of the nature and scope of the treatment provided by each of the health professionals in question and further, that the defendant had the opportunity to summons these witnesses.

(a) The relevant legal principles

[168] It has long been the case that an adverse inference may be drawn by a court against a party who does not call a material witness, over whom that party has exclusive control, and does not provide an explanation for doing so. The theory is that failure to call such a witness amounts to an implicit admission that the evidence of that particular witness would be somehow contrary to the party's case and would therefore compromise the interests of that party: see *Levesque v. Comeau*, [1970] SCR 1010.

[169] While the decision to draw an adverse interest is squarely within the discretion of the court, it should not be done where there is a legitimate explanation for not calling the witness, and the witness is equally available to both parties. The Court should also consider whether the witness in question actually has key evidence to provide and whether that individual is the best person to provide the evidence in question: see *Parris v. Laidley*, 2012 ONCA 755.

[170] This issue was thoroughly considered by this court in the case of *Bishop-Gittens v. Lim*, 2015 ONSC 3971 where McKelvey J. set out a three-part test to be applied by a court when considering whether to draw such an inference. That test is as follows:

- (a) The witness must have key evidence to provide.
- (b) There can be no adequate explanation for the failure of the party to call a witness.
- (c) The witness must be within the exclusive control of the party against whom the adverse inference is sought to be drawn.

When a party fails to call a witness in such circumstances then the adverse inference is quite properly drawn.

[171] The defendant in the *Bishop-Gittens* case argued that the plaintiff's failure to produce its treating physician and its employer should lead to an adverse interest because those witnesses were under the plaintiff's exclusive control. Justice McKelvey disagreed with that position. He noted that the discovery obligations in personal injury matters require that treatment records be disclosed to all parties in the action well in advance of the trial, which was done in that case. Defence counsel in that matter therefore had full access to all of the relevant records, knew the identity of the potential witnesses, and would have been able to subpoena those witnesses himself had he felt it necessary.

[172] Justice McKelvey went on to explain that it has become common practice in cases of this nature for defence counsel to summons all treating physicians as a standard part of their preparation for trial.

[173] Such could be seen as an implicit recognition by the defence of the fact that such witnesses are not within the exclusive control of the plaintiff but rather, are available for all parties to summons and to call to give evidence.

[174] There are some good policy considerations behind this approach. To do otherwise would mean that a plaintiff would have to call each, and every medical professional involved in the case for fear that the defence would ask the court to draw an adverse inference. That of course would significantly lengthen trials and thereby increase expense, all while doing little, if anything, to actually advance the case. That would be counter-productive and, in my view, would make it even more challenging for litigants in Canada to access justice. As the Supreme Court of Canada noted in *Hyrniak v. Mauldin*, 2014 SCC 7, ensuring access to justice is the greatest challenge to the rule of law in Canada today.

(b) Application of the relevant legal principles to the facts of this case

[175] For the following reasons I declined to draw an adverse inference against the plaintiff for not calling these witnesses:

- The clinical records and notes of the various health professionals referred to by the defendant, including the plaintiff's family doctor, his treating psychologist, his treating chiropractor, and his treating physiotherapist, were all supplied to the defendant at the discovery stage of the process.
- Those clinical records and notes were also made exhibits and are contained in the Joint Document Brief.
- The defendant therefore had full access to all of the relevant records and to the identity of each of the relevant witnesses at its disposal.
- Dr. Su, who was the plaintiff's primary treating physician, gave lengthy oral evidence about his treatment of the plaintiff. During the course of his evidence he touched on many aspects of the treatment provided by the other health professionals involved in the case.
- The plaintiff chose to rely on the evidence of Dr. Su as calling the other witnesses would have resulted in lengthy and repetitive evidence which would have been of little value to this Court.
- The defendant had every opportunity to summon each of the other health professionals to testify but apparently chose not to do so.

[176] In conclusion, I am satisfied that the circumstances of this case do not fit within the test set out by McKelvey J. in the *Bishop-Gittens* case. The witnesses do not have key evidence to provide, there is an adequate explanation for the failure of the plaintiff to call these witnesses, and the witnesses were not within the exclusive control of the plaintiff.

ISSUE #4: Has the plaintiff established on a balance of probabilities that he meets the exceptions set out in s. 267.1(3) and (4) of the Insurance Act, aka ‘the threshold’?

[177] At the close of the evidentiary phase of this case the defendant brought a motion for an order that the plaintiff has not met the minor injury threshold set out in s. 267.1 of the *Insurance Act*.

(a) The Relevant Law and Legal Principles

[178] Section 267.1 sets out certain exceptions that the plaintiff must meet in order to be successful in his claim for non-pecuniary damages against the defendant. The section provides for liability where the injured person has died, or has sustained a serious disfigurement, or a serious impairment of an important physical, mental, or psychological function.

[179] The following is a summary of the criteria set out in Section 4.2 of Ontario Regulation 461/96 that define permanent serious impairment:

1. It must substantially interfere with a person's ability to perform the necessary functions of their regular employment, career, or activities of daily living.
2. It must interfere with the necessary responsibilities of their employment, career, activities of daily living, or their own self care; and
3. It must have been continuous from the time of the accident and must, based on medical evidence and subject to the person reasonably participating in the recommended treatment of the impairment, be expected not to substantially improve.

[180] Section 4.3 of the regulation sets out the evidentiary requirements to meet the threshold. It requires that a physician must explain the nature of the impairment, its permanence, the specific function that is impaired, and the importance of that function to the person.

Meeting the Threshold

[181] In order to meet the threshold a plaintiff must meet the test set out in the seminal case on the issue, *Meyer v. Bright*, (1993) 15 O.R. (3d) 129 (Ont. C.A.). That test consists of the following three steps:

1. Has the injured plaintiff sustained permanent impairment of a physical, mental, or psychological function?
2. If the answer to question number one is yes, is the function which is permanently impaired an important one?

3. If the answer to question number two is yes, is the impairment of the important function serious?

The test requires a determination be made in each case as to whether the injured person falls within one or more of the statutory exceptions to the general immunity provided by the *Insurance Act*, being death, permanent serious disfigurement, or permanent serious impairment of an important bodily function.

1. The impairment must be permanent

[182] The word “permanent” in this context has been held to mean “a weakened condition lasting into the indefinite future without any end limit, as opposed to one predicted to have some defined end.” See: *Bos v. James*, [1995] O.J. No. 598, *Morrison v. Gravina*, [2001] O.J. No. 2060, and *Valentine v. Rodireguez-Elizalde*, 2016 ONSC 3540.

2. The impairment must be of an important function

[183] An important function is one that plays a major role in the health, general well being, and way of life of the injured plaintiff. This involves a subjective analysis, as a Court must consider the importance of the bodily function in issue as it relates to the particular individual who is affected by the impairment. Both the injured person, and the effects of the particular impairment on that person's way of life, must be considered as a whole. If the bodily function in question is important to the particular plaintiff, then the impairment is an important one. In reaching that determination the court must consider all aspects of the injured person's life and the degree to which previous activities have been compromised by the injuries in issue. See: *Meyer v. Bright*, supra, *Vandenburg v. Montgomery*, [1999] O.J. No. 2789, and *Arteaga v. Poirier*, 2016 ONSC 3712

[184] If the impairment affected important aspects of the injured person's life, or if the activities that are curtailed are essential to the injured person's daily life, then the impairment is considered to be “important”. In other words, is the impaired function one that plays a major role in the health, general well being, and way of life of the particular plaintiff? *Knudsen v. Tyckj*, [1994] O.J. No. 2763, and *Valentine v. Rodireguez-Elizalde*, supra.

[185] Diverse activities such as recreational pastimes, participation in and attendance at church activities, or care for children and other family members, have all been found to be “important” aspects of daily life. An injury that restricts or prevents an injured person from performing such activities is said to constitute a serious injury. *Vandenburg v. Montgomery* supra and *Morrison v. Gravina*, supra. Other such activities include the following:

- household repairs
- domestic chores
- employment

- attending concerts and the theatre
- fishing
- bridge playing
- the lack of energy which prevents full time work and the ability to engage in “normal activities of daily living”
- walking and dancing
- intimate relations
- socializing with others

3. Is the impairment of the important function serious?

[186] In determining whether or not the impairment is “serious” it is necessary to proceed on a case-by-case basis. There is no absolute formula to determine what is serious however, generally a serious impairment is one which causes substantial interference with the ability of the injured person to perform their daily activities or to continue their regular employment. In order to qualify as serious an impairment must be serious - no more and no less: see *Meyer v. Bright*, supra at paras 33-34.

[187] In the case of *Brak v. Walsh* 2008 ONCA 221, the Ontario Court of Appeal articulated some factors that are felt to meet the “serious” component of the threshold test:

The requirement that the impairment be ‘serious’ may be satisfied even though plaintiffs, through determination, resume the activities of employment and the responsibilities of household but continue to experience pain. In such cases it must also be considered whether the continuing pain seriously affects their enjoyment of life, their ability to socialize with others, have intimate relations, enjoy their children, and engage in recreational pursuits.

[188] Notably, it has been held that a return to work, even where the person injured begins to earn more money, will not necessarily result in a finding that the plaintiff does not have a serious impairment. A plaintiff’s ability to return to work and to continue employment is not fatal to a threshold motion. The Court must examine the totality of the evidence, including the impact of the impairment on the plaintiff’s ability to continue working, as well as the converse, being the impact of continued work on the plaintiff.

[189] Moreover, where a plaintiff continues to work notwithstanding significant pain it may be as a result of that particular plaintiff’s work ethic as opposed to an admission of freedom from

pain. This is particularly so when a plaintiff is left exhausted by their workday such that he or she is unable to ability to enjoy their home life and other, previous daily activities outside the workday: see *Arteaga v. Poirier*, supra.

(b) Application of the Relevant Legal Principles to the Facts of This Case

(i) Has the plaintiff sustained a permanent impairment?

[190] Doctor Su testified that the plaintiff's injury is permanent and chronic.

[191] Doctor Axelrod also testified the injury is permanent. He testified that the plaintiff participated in a number of treatments, including cortisone injections, physiotherapy, chiropractic treatment and exercise. Those treatments did not alleviate the condition in any meaningful way.

[192] Doctor Kumar was also of the opinion that the injury is permanent and chronic.

[193] More than six years has elapsed since the accident. The plaintiff is now 62 years of age and continues to have pain in his elbow, despite the attempts he has made to try to mitigate his injury. The man who once was actively engaged in his chosen profession, as well as in a wide assortment of recreational activities, has morphed into an individual who is able to attend his place of employment on a daily basis but is unable to engage in the more physical activities of that employment, activities that he previously enjoyed and preferred. He is also no longer engaged in the recreational activities that he used to enjoy and is now, in his leisure time, effectively relegated to his recliner in front of his television.

[194] Based on the evidence in this matter that I accept, I find that the plaintiff meets the condition for "permanent" impairment.

(ii) Has the plaintiff sustained an impairment of an important bodily function?

[195] As a result of the muscle tear the plaintiff has suffered a declining ability to make full use of his left arm. He has also experienced atrophy and muscle wasting such that his left arm is now 3 centimeters smaller in mass than his right arm.

[196] All three of the plaintiff's doctors testified that the muscle wasting was a result of disuse caused by pain interfering with the ability to actually use the arm.

[197] Doctor Axelrod and Doctor Su both suggested the elbow injury and the atrophy are indicative of a serious issue.

[198] The plaintiff has been an auto mechanic his entire life. He developed that interest in his teen years and has enjoyed his daily work in that field ever since. He enjoyed an excellent reputation in the community generally and among his clientele for being a good mechanic.

[199] The plaintiff's employees all gave evidence about the impact that the injury has had on the plaintiff's ability to engage in the daily tasks of an auto mechanic. Simply put, he is unable to engage in those tasks and now works in a strictly administrative capacity.

[200] The plaintiff's life outside the shop floor has also been affected in as much as he is unable to do the housekeeping and outdoor maintenance that he previously enjoyed doing, nor is he able to engage in his lifelong handyman activities for himself or for his neighbors. What I found most striking was the plaintiff's testimony that he has difficulty even taking out the garbage.

[201] The plaintiff is also no longer able to enjoy his lifelong affinity for golf, fishing, or boating.

[202] In view of all of those circumstances, I find that the plaintiff has sustained an impairment of an important bodily function as he is unable to engage in his former work, home, and recreational activities.

(iii) Has the plaintiff sustained a serious impairment?

[203] For the following reasons I find that the plaintiff has sustained a serious impairment:

- As a result of the disruption to his regular routine caused by the accident the plaintiff has developed anxiety and mood issues.
- His daily routine has been reduced to a sedentary lifestyle from his former active and energetic work life and private life.
- His traditional active participation in the daily work of an auto mechanic at his shop has been curtailed to an administrative role only.
- The plaintiff can no longer perform the physical labour required for his chosen career and he has had to hire replacement workers to complete work that he could not do himself.
- As the plaintiff must now rely on employees to complete the work required by his clientele, he is at the mercy of an uncertain labour market. He has lost employees to car dealerships who have higher rates of pay. He depends on his business for the support of himself and his family however he is unable to do that work himself.
- Both of his attending physicians have concluded that the accident interfered with the plaintiff's ability to do his chosen work. They have also diagnosed him with an adjustment disorder with mixed mood and anxiety due to the fact of the collision and its consequent impact on his work and financial life.
- As I have noted elsewhere in these reasons, the plaintiff has persevered despite the limitations imposed on him by his injury. However, that perseverance has clearly taken a toll on the plaintiff's ability to enjoy his work and home life, and, it should also be said, on his family's ability to enjoy him.

- The plaintiff can no longer engage in the recreational activities that he used to enjoy.
- He can no longer perform basic tasks associated with the upkeep of the family home.

(iv) Conclusion on the threshold issue

[204] I find that the plaintiff has suffered a permanent, serious impairment of an important physical, mental, or psychological function and a permanent serious disfigurement.

[205] I therefore find the plaintiff's case meets the threshold requirements set out in s. 267 of the *Insurance Act*.

[206] The defendant's threshold motion is therefore dismissed.

CONCLUSION/SUMMARY

[207] Judgment for the plaintiff as follows:

1. General damages for pain and suffering in the amount of \$75,000.
2. Pecuniary damages for loss of competitive advantage in the amount of \$571,595.
3. Pecuniary damages for loss of R.E.R. revenues in the amount of \$212,970.
4. Past and future costs of care in the amount of \$9,903.
5. Damages for loss of housekeeping and handyman abilities in the amount of \$25,000.
6. Pre and post judgment interest in accordance with the provisions of the *Courts of Justice Act*.

[208] The total damage award is therefore \$894,468.00.

COSTS

[209] In the event the parties cannot agree on costs they may address me in writing on the issue. Each party's costs submissions shall be limited to 5 pages double spaced, plus appropriate attachments which shall include a bill of costs. The plaintiff shall serve and file his costs submissions on or before June 3, 2022. The defendant shall file its materials on or before June 10, 2022.



C. M. Smith, J

Released: May 16, 2022

CITATION: *Herrington v. Brewer et al*, 2022 ONSC 2852
COURT FILE NO.: 3076/17
DATE: 20220516

2022 ONSC 2852 (CanLII)

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:

PAUL A. HERRINGTON

Plaintiff

– and –

WALTER W. BREWER, ONTARIO CORPORATION
NUMBER 1623687 operating as NEW HORIZONS
CAR AND TRUCK RENTALS LTD., JENNIFER M.
STEWART and ONTARIO
CORPORATION NUMBER 624252 operation as
WESTERN ASSURANCE COMPANY

Defendants

REASONS FOR JUDGMENT

Smith, J

Released: May 16, 2022